

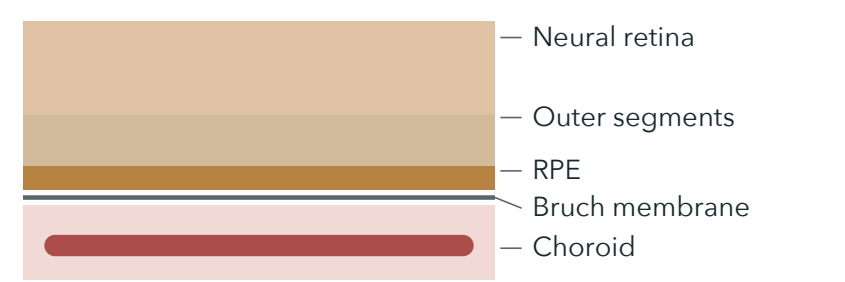
AMD at the RPE-Bruch interface

TYPE B | Locate the deposit or vessel before naming the process.

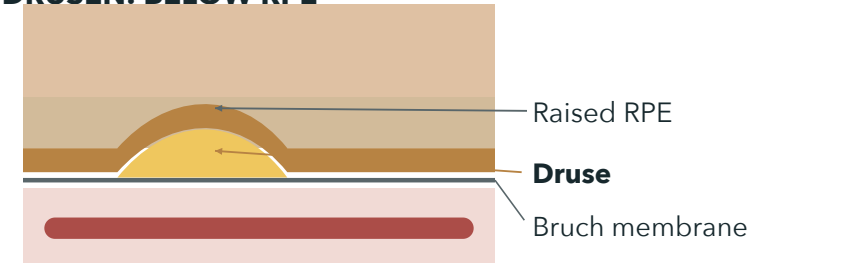
Drusen occupy the outer interface

Layer map: normal interface and drusen

NORMAL OUTER RETINAL INTERFACE



DRUSEN: BELOW RPE



Drusen are extracellular deposits between basal RPE and Bruch membrane. They are not intraretinal hard exudates and do not automatically mean neovascular AMD.

Compare yellow deposits

Drusen	Hard exudates
Below RPE, above Bruch membrane	Within the retina
Associated with ageing and AMD	Derived from retinal vascular leakage
Outer retinal support interface	Lipoprotein deposition around leaking vessels

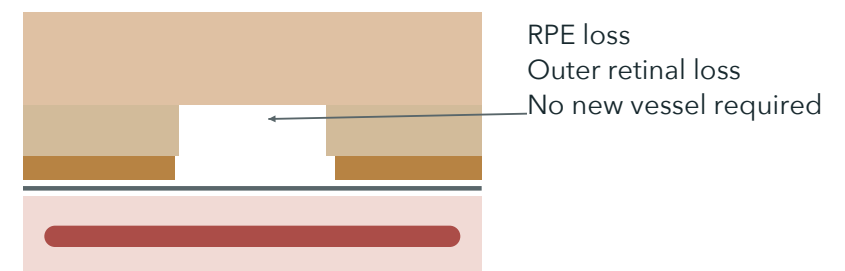
Why the macula matters

RPE, Bruch membrane and choriocapillaris support outer retinal function. Macular injury produces central blur, distortion or a central scotoma; peripheral navigation can remain useful.

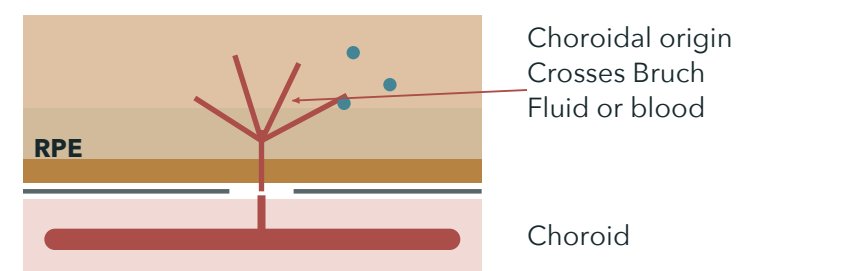
Atrophy or a leaking vessel network

Two advanced macular injury patterns

GEOGRAPHIC ATROPHY



NEOVASCULAR AMD



Geographic atrophy loses RPE and photoreceptors. Neovascular AMD adds abnormal choroidal vessels into sub-RPE or subretinal compartments.

Dominant change → visual effect

Non-neovascular atrophy	Neovascular AMD
Progressive RPE/outer retinal loss	Abnormal choroidal vessel growth
No new vessel required	Vessels cross defects in Bruch membrane
Central dysfunction from tissue loss	Leakage/bleeding distort macular structure
Drusen may coexist	Drusen may precede the new vascular lesion

Avoid one common shortcut

Retinal surface neovascularisation in diabetes and choroidal neovascularisation in AMD have different origins and planes. New central distortion with fluid or blood suggests structural macular disruption, not regained or lost accommodation.